NCCP 00401 · clinician draft

Alectinib monotherapy

> **Confirm your alectinib treatment setting** This guide covers alectinib alone for ALK-positive lung cancer. Confirm whether it is adjuvant treatment after complete surgery, first treatment for advanced cancer, or treatment after crizotinib. These settings have different evidence and treatment lengths.
>
> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Alectinib daily oral treatment

Active medicines: alectinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Alectinib is minimal-to-low oral emetic risk. Treat nausea with prescribed as-required medicine if needed. Routine preventive anti-sickness treatment and infusion premedication are not required; your team selects a suitable rescue medicine.
- The usual starting dose is 600 mg twice daily: four 150 mg capsules each time, taken with food. Use your own prescription, which may specify a lower dose. Swallow capsules whole; do not open or dissolve them. If a dose is missed, take it only if the next dose is more than 6 hours away. If the next dose is due within 6 hours, skip the missed dose. Do not take two doses together. If you vomit after a dose, take the next dose at its usual time; do not take an extra dose.
- Keep liver, CPK and blood-count appointments. Muscle symptoms, jaundice, dark urine, unusual fatigue, cough or breathlessness need prompt review even between appointments.
- Tell the team about dizziness, fainting and medicines that slow the pulse. Check new prescriptions, supplements, grapefruit and Seville oranges with pharmacy; interactions may require closer monitoring rather than an automatic dose change.
- Avoid prolonged sunlight during treatment and for at least 7 days after stopping. Use broad-spectrum UVA/UVB sunscreen and lip balm SPF 50 or higher.
- Women who could become pregnant need highly effective contraception during treatment and for at least 5 weeks afterwards. Men with female partners who could become pregnant need highly effective contraception during treatment and for at least 3 months afterwards. Oral contraceptive effectiveness may be reduced: ask for a suitable method. Do not breastfeed during treatment. Discuss fertility before treatment and contact the team promptly if pregnancy occurs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00401 v6 · p4; oral classification Tables 5 and 8 · NCCP oral minimal–low risk: as-required antiemetics only; NCIS V8 checked separately

Sources checked 2026-10-29 · SACTCheck v0.84.24 · Regimen-level drafts; prescriber and oncology-pharmacy verification required